

RZLT pitch deck

PITCH — RZLT — VERDICT

PROPOSED — PENDING ADJUDICATION — No position. We do not own Rezolute today, and we should not start one at \$5.09, because the company's own trial registry says the event that decides this stock happens roughly a year later than the company's guidance says it does.

What we believe

Rezolute is a one-drug company. The drug is ersodetug, an antibody for hyperinsulinism — a condition where the body makes too much insulin and blood sugar crashes dangerously low. There are two versions of the disease the company is chasing: a congenital form (children born with it) and a tumor-driven form (adults whose tumors secrete insulin or an insulin-like factor).

Three things we believe, in order of confidence:

1. The near-term catalyst calendar is not real. The company has told investors twice — in its May 12, 2026 quarterly release and again in a June 2, 2026 8-K — that topline results from the upLIFT trial (the tumor version, 16 patients, single-arm) are expected in the second half of 2026. The same company's own entry on ClinicalTrials.gov for that trial, NCT06881992, was updated on August 17, 2026 — four days before we looked — and lists an estimated primary completion date of **September 2027**. That is a twelve-month gap between what the company tells investors and what it tells the trial registry. Enrollment corroborates the registry, not the guidance: 13 sites across 5 countries have produced 8 patients since April 2025, roughly 0.6 patients a month. At that rate the trial fills around mid-2027, and then needs an 8-week treatment period and a database lock on top of that.

2. The valuation is roughly fair, not obviously wrong. Our own bear case built a bottom-up value for the tumor business assuming the drug is fully funded and certain to be approved — a deliberately generous setup — and got \$4.59 a share. That number turned out to be built on two separate conservative errors (detailed below). Corrected, the same model on the same patient-count assumptions gives **\$6.37 to \$9.09 a share**. Against a \$5.09 tape, the market is implying roughly a 60% chance of success on the corrected central case, and something close to 100% on the most pessimistic parameterization. That is a fair price, not a gift and not a bubble.

3. The size of the tumor market is much smaller than the story being told. The number that circulates around this stock is 3,000 patients. The company's global expanded-access program — which gives the drug free to patients outside a trial, across the US and Europe — has accumulated “more than a dozen” patients, with published data on nine. A global Phase 3 needing only 16 people stood at 8. Our bull-side reviewer correctly noted that upLIFT enrolls a narrow slice (hospitalized patients on IV glucose), so 8-of-16 does not directly measure the total market. But the expanded-access program has

no such restriction and still can't find people. Published epidemiology puts the treatable pool somewhere around 300–450 patients in the US, not 3,000.

Net: the drug may well work and the price may well be fair — but we cannot buy an event we cannot date, and the sponsor's own registry says we can't date it.

What the market believes

Ten analysts cover the stock. The average price target is \$7.67, the median \$6.00, the low \$4.00, and the high \$14.00. Next fiscal year revenue consensus is \$2.3 million — essentially nothing, so every dollar of that target is the drug's future value (stockanalysis.com/stocks/rzlt/forecast).

The market believes: (a) the tumor indication is a real, near-term approval path; (b) the H2-2026 topline guidance is credible; (c) the population is large enough to matter — the sell-side framing is a 3,000-patient opportunity; and (d) the December 2025 failure of the sunRIZE trial in the congenital indication was a measurement problem, not a drug problem.

Short interest is about 10.5% of the float, so there is a meaningful contingent that disagrees. Average daily volume is about 1.67 million shares, roughly \$8 million a day, so we could enter or exit at our size without difficulty. Four institutional ownership filings in the last three months are all passive index-tracker filings, not activists.

The stock is down 55.6% from its 52-week high of \$11.46 and up 3.8x from its 52-week low of \$1.07. Both of those are within the last twelve months. This has been an extremely violent tape.

Why we might have edge

The registry-versus-guidance gap is a checkable, dated, primary-source contradiction that nobody appears to have priced. Both of our reviewers pulled the same public documents. Only one of them opened ClinicalTrials.gov. The company says H2 2026; its own registry entry, freshly touched, says September 2027. This is the kind of finding that exists in public but requires someone to go look at two venues and diff a number. It is not in any analyst note we found, and the \$4.00 street low is built on a bear valuation case, not a bear calendar case.

Second: the enrollment arithmetic is a market-size readout the sell-side hasn't done.

Nobody appears to have divided the expanded-access patient count into the claimed 3,000-patient market. When a company gives a drug away free, globally, to a disease with no other treatment, and gets a dozen takers, that tells you something about how many patients exist.

Third: the runway and the readout now collide. The company had \$120.2 million in liquid assets at March 31, 2026, and used about \$17.2 million a quarter over the prior nine months, with the burn rising. That is roughly seven quarters, exhausting around Q4 of calendar 2027 — which is exactly when the registry says the trial reads out. There is no debt and the \$50 million Jefferies at-the-market equity program was terminated in October 2025 with nothing drawn. Meaning: the company will very likely need to raise money before or into its own catalyst, and it has no standing facility to do it quietly.

Why it might be priced in

Being honest about the other side:

The 12-month registry gap may be nothing. Registry completion dates are routinely stale padding that sponsors never bother to pull in. The counter-argument is that this record was touched on August 17, 2026 — the sponsor was in the file and left the date alone — and that the observed enrollment velocity independently supports the later date. But “sponsors are lazy about registry dates” is a real and common phenomenon, and we have not disproven it here.

The market has already discounted a lot. Down 55.6% from the high. A \$4.00 street low target. 10.5% short interest. The bear case is not undiscovered — it is partly published. Our valuation work does not put us below the street; it puts us inside the street’s range, between the \$6.00 median and the \$14.00 high. We do not have a valuation disagreement with the consensus.

The regulatory concern is softer than it first looked. Our bear reviewer’s headline kill was that the FDA agreed to the small 16-patient single-arm design in August 2025 explicitly because the separate sunRIZE trial “would serve as confirmatory clinical evidence” — and sunRIZE then missed its primary endpoint on December 11, 2025 (45% vs 40% on placebo, not statistically significant). The argument was that the deal lost its consideration and the company has gone quiet about it. On review that is overstated: the May 12, 2026 release does restate the trial design and the H2-2026 timing, and at a March 17, 2026 Type B meeting the FDA reportedly acknowledged the limitations of the self-monitored blood-glucose measurements the company blames for the sunRIZE result. A missed primary endpoint does not zero out a trial’s usefulness as supporting mechanistic evidence — the drug arm did cut events by about 45%. This is an unresolved open risk, not a proven collapse.

We are assigning zero value to the congenital indication. That is conservative to the point of being wrong. It has Breakthrough Therapy designation, an active FDA dataset evaluation, and a larger population. Any value there is upside we are not counting.

What we checked ourselves

Claim	How checked	Found	Confirmed?
Share count / dilution overhang	10-Q cover page and company Q3 FY2026 release	96,292,331 shares outstanding at 2026-05-08 plus 8.2M pre-funded warrants. The dilution happened in an April 2025 \$90M offering — eight months <i>before</i> the drawdown, so it is not a live overhang	Yes on substance
Our bear reviewer’s “correction” of the share count	Company’s own release, line “shares used to compute basic and diluted net loss per share” = 104,040 thousand	OUR BEAR BENCH WAS WRONG. It accused the original brief of comparing a weighted-average figure to an outstanding count. The 104.0M figure <i>is</i> the weighted-average diluted line — the same concept. The comparison was correct all along	Bench error — overturned

Claim	How checked	Found	Confirmed?
Fully-diluted share count used in the valuation	Rebuilt with option-exercise proceeds included	OUR BEAR BENCH WAS WRONG AGAIN. It counted 12.4M options and 2.3M RSUs as new shares but omitted the cash those options bring in when exercised — a one-sided calculation. Correct figure is roughly 110M, not 120M. This error alone cost about 13% of the per-share value	Bench error — overturned
“Our value is below the street’s lowest target”	Simple arithmetic	OUR BEAR BENCH WAS WRONG A THIRD TIME. Its own cited street low was \$4.00. Its own computed value was \$4.59. \$4.59 is above \$4.00. The claim that we sit under the entire street was arithmetically false	Bench error — overturned
“The company stopped restating the regulatory path after sunRIZE failed”	Read the 2026-05-12 and 2026-06-02 releases directly	PARTLY WRONG. The trial design and timing <i>are</i> restated on 2026-05-12. What is not restated is the specific evidentiary predicate — which trial now serves as confirmatory evidence. That is a narrower and weaker point than “silence”	Bench error — reduced from fatal to open risk
The FDA “confirmatory clinical evidence” quote itself	2025-09-02 company press release	Verified verbatim — but read via a third-party mirror, because both the wire service and the company’s own investor-relations page timed out	Verified, second-venue only
upLIFT trial timing	ClinicalTrials.gov NCT06881992, pulled directly	Estimated primary completion September 2027 ; status recruiting; 13 sites, 5 countries; record last updated 2026-08-17	Confirmed — and this became the decisive finding
Enrollment velocity	Company 8-K 2026-06-02	8 of 16 patients since 2025-04-16 $\approx$ 0.6 patients/month	Confirmed
Cash runway	10-Q for the quarter ended 2026-03-31 (SEC filing)	\$120.2M liquid; nine-month operating cash use \$51.5M; no debt; ATM terminated Oct 2025 undrawn → exhaustion ~Q4 CY2027	Confirmed by both benches
Market size	Published epidemiology, bottom-up	Malignant insulinoma ~6% of an 0.7–4 per million annual incidence → 14–80 US cases/yr; non-islet-cell tumor hypoglycemia ~1 per million/yr → ~335 US cases/yr, with short survival driven by the underlying cancer. Treatable prevalent pool ~300–450 US	Confirmed as a range; not sourced from the company
The 4.0x peak-sales multiple our bull bench used	Judgment call, not a source	OUR BULL BENCH ALSO PUSHED ITS THUMB ON THE SCALE. It raised the multiple from 3.0x to 4.0x while simultaneously correcting the share count downward. Both changes move value up. The share-count fix is a documented error correction; the multiple change is an opinion swap with no citation. Roughly half of the value uplift is assumption, not error-correction	Not verified — flagged as assumption

Claim	How checked	Found	Confirmed?
Next earnings date	Pack, yfinance-derived	2026-09-16 — not confirmed by any company announcement. Corroborated only by the analogous FY2025 release on 2025-09-17	Unconfirmed
Insider activity	Attempted Form 4 pull for 2026-07-07 (three filings)	Could not read them. SEC returned server errors. Three insider transactions in one day, contents unknown	NOT CHECKED — open gap
Expanded-access program patient outcomes	Attempted NANETS conference abstract	Blocked (403). Patient count and how long patients stay on drug are both unverified — and both are load-bearing for the market-size argument	NOT CHECKED — open gap

Summary of our own error rate on this name: our bear reviewer made three separate arithmetic or basis errors, all of which pushed toward rejection. Our bull reviewer corrected them, then introduced one unsourced assumption pushing the other way. The rejection survives, but for a completely different reason than the one originally given.

PROPOSED ENTRY BANDS

There is no entry today. The following are the conditions under which we reopen.

The math behind these numbers: value per share = (peak global sales × multiple ÷ 1.405 for three years of discounting at 12%) + ~\$103M net cash, divided by ~110M shares. Net cash alone is about \$0.94/share. On the most conservative version (3.0x multiple, 300 US patients), the risk-bearing part of the business is worth about \$4.07/share if the drug is certain to work. On the corrected central version (4.0x, ~375 patients), it is about \$6.79/share.

At today's \$5.09, the market is paying about 61% of the corrected central prize, and about 102% of the most pessimistic prize. In plain terms: **at this price you are already assuming the drug works.**

Reopen band 1 — calendar fix (the primary one). If the company announces upLIFT is fully enrolled at 16 of 16 *with a dated topline*, or if the ClinicalTrials.gov primary-completion date is pulled into 2026 or the first half of 2027, then: - **First tranche: buy up to \$7.05.** That is our bear bench's own conservative funded value at 450 patients — we would be paying no more than a fully-funded, zero-risk price under pessimistic market-size and multiple assumptions. - **Size: 0.4% of the book.** Small, because a single-asset biotech with a binary readout and a required financing has an estimated –80% downside if the trial fails. - **Second tranche: an additional 0.3% only after the financing is done and priced.** Do not add before the raise. This company must raise money before its own readout and has no standing facility to do it quietly. - **Hard cap: 1.0% of the book, ever, pre-approval.**

Reopen band 2 — price fix (valuation-only, no calendar fix required). - **Below \$3.00**, a starter becomes defensible on price alone. At \$3.00 you are paying about 50% probability of success against the *most pessimistic* version of the prize (3.0x multiple, 300 patients, zero value for the congenital indication). Size 0.35% of the book. - **Below \$2.36**, implied probability of success falls under 35% on that same pessimistic case. Size up to 0.75%. - **Note the honest problem with this**

band: any move to \$3.00 would probably be *caused* by the trial slipping or a punitive financing, in which case the probability of success genuinely is lower and the band should be re-derived rather than mechanically executed. Do not buy a falling knife on a stale model.

Do not buy between \$3.00 and \$7.05 without a calendar fix. That is the zone we are in now, and it is the zone where we are paying for an event we cannot date.

Exits/trim: Not applicable — no position.

Catalyst dates: - **2026-09-16** — expected fiscal-year earnings release. **Not confirmed by the company.** Low information for the thesis itself, but carries real risk: this is where management either repeats “second half of 2026” or quietly changes the language. A change in that language is the trade. - **“Second half of 2026”** — company-guided upLIFT topline. Undated. We do not believe it. - **September 2027** — ClinicalTrials.gov estimated primary completion for NCT06881992. Registry value, an estimate, but from the sponsor and freshly updated. - **~Q4 calendar 2027** — projected cash exhaustion. Note this lands on top of the registry readout date. A financing must therefore happen before the catalyst.

ARMED SENSORS

(armed provisionally with this deck; session adjudication ratifies)

- price alert: fires below 2.36 (deck-provisional: Implied probability of success under 35% on the same pessimistic case; size up to 0.75% of book, but r)
- filing watch: 8-K,PR until 2027-02-17 (deck-provisional: upLIFT enrollment reaching 16/16 with a dated topline — triggers Reopen Band 1, the primary entry cond)
- dated pack RZLT|2026-09-16: RZLT: Expected fiscal-year earnings release — yfinance-derived, NOT confirmed by the company; watch for any change to the ‘second half of 20
- dated pack RZLT|2026-12-31: RZLT: End of the company-guided ‘second half of 2026’ window for upLIFT topline results — we do not believe this date will be met (date conf
- dated pack RZLT|2027-09-30: RZLT: ClinicalTrials.gov estimated primary completion for NCT06881992, record last updated 2026-08-17 — twelve months after company guidance
- dated pack RZLT|2027-12-31: RZLT: Projected cash exhaustion at ~\$17.2M/quarter from \$120.2M at 2026-03-31 — lands on top of the registry readout, so a financing must pr

What remains unverified

Stated plainly, because several of these are load-bearing:

1. **Whether the FDA still accepts the 16-patient single-arm design as sufficient** for a tumor-indication filing now that the trial it originally named as confirmatory evidence has failed. No company or agency statement addresses this in eight months of disclosure. We do not know if this is a problem or a non-issue.

2. **The expanded-access patient count and how long patients stay on the drug.** The abstract was blocked. Treatment duration is a direct multiplier on our revenue estimate — if patients stay on for two years instead of one, every value figure in this deck roughly doubles.
3. **The \$500,000/year net US price assumption.** Not verified anywhere. It is our number, not the company's.
4. **The 300–450 prevalent-patient range.** Assembled from published epidemiology, not from the company. The company has never disclosed a sourced patient count with a duration assumption. Our bull reviewer flagged that the trial's narrow inclusion criteria mean enrollment does not directly measure market size, and that objection stands.
5. **Whether the September 2027 registry date is meaningful or stale padding.** We argued it is meaningful because the record was touched on 2026-08-17 and enrollment velocity agrees. That is inference, not proof.
6. **Any value at all in the congenital indication.** We assigned zero. That is almost certainly too low, but we have no basis for a number.
7. **The 4.0x peak-sales multiple.** Our own bull reviewer's choice, unsourced. The bear's 3.0x is equally unsourced. Every value figure in this deck is sensitive to a number neither bench justified.
8. **The 2026-09-16 earnings date.** Third-party derived, uncorroborated by the company.
9. **The 2025-09-02 FDA-alignment quote.** Verbatim, but read from a mirror because the company's own investor page and the wire service both timed out. We should re-read it from the primary source.
10. **Live intraday pricing.** All prices in this deck anchor to a single snapshot at \$5.09; we could not re-quote.

Conviction that no-position is right: 8/10. Conviction that the reason is the calendar rather than the valuation: 8/10. Conviction in any specific value-per-share number in this deck: low — our own two reviewers disagreed by 39% on the same model with the same inputs, and both of them made errors.

```
{"price_gates": [{"level": 3.00, "direction": "below", "basis": "Implied probability of success falls to ~50% against the most pessimistic funded prize (3.0x multiple, 300 US patients, zero credit for the congenital indication) – starter defensible on price alone at 0.35% of book"}, {"level": 2.36, "direction": "below", "basis": "Implied probability of success under 35% on the same pessimistic case; size up to 0.75% of book, but re-derive the model first since a move here likely reflects a real deterioration"}, {"level": 7.05, "direction": "above", "basis": "Bear bench's own conservative fully-funded, zero-risk value at 450 US patients – above this we are paying more than certainty is worth under pessimistic assumptions; do not chase"}], "event_gates": [{"forms": ["8-K", "PR"], "why": "upLIFT enrollment reaching 16/16 with a dated topline – triggers Reopen Band 1, the primary entry condition"}, {"forms": ["8-K", "PR"], "why": "Any company or FDA statement naming what now serves as confirmatory clinical evidence for the tumor-hyperinsulinism filing after sunRIZE failed – resolves the largest open regulatory question"}, {"forms": ["S-3", "424B5", "8-K"], "why": "Financing: the company must raise before its own readout and terminated its ATM undrawn in Oct 2025 – a raise is a known adverse event, do not average down into an unpriced one"}, {"forms": ["4"], "why": "Three insider transactions on 2026-07-07 remain unread due to SEC server errors; ongoing insider activity is unmonitored"}, {"forms": ["10-Q"], "why": "Quarterly burn rate against the ~Q4 CY2027 exhaustion estimate, which collides with the registry readout date"}, {"forms": ["SC 13D"], "why": "Current holders are all passive index filers; an activist filing changes the ownership picture materially"}], "catalyst_dates": [{"date": "2026-09-16", "what": "Expected fiscal-year earnings release – yfinance-derived, NOT confirmed by the company; watch for any change to the 'second half of 2026' topline language", "confirmed": false}, {"date": "2026-12-31", "what": "End of the company-guided 'second half of 2026' window for upLIFT topline results – we do not believe this date will be met", "confirmed": false}, {"date": "2027-09-30", "what": "ClinicalTrials.gov estimated primary completion for NCT06881992, record last updated 2026-08-17 – twelve months after company guidance, corroborated by 0.6 patients/month enrollment velocity", "confirmed": false}, {"date": "2027-12-31", "what": "Projected cash exhaustion at ~$17.2M/quarter from $120.2M at 2026-03-31 – lands on top of the registry readout, so a financing must precede the catalyst", "confirmed": false}], "immediate_entry": null}
```